

BluePaw Emergency & Specialty Center

4500 Research Blvd, Suite 200, Austin, TX 78759
Tel: (512) 555-9100 | Vet License: TX-DVM-061234
Attending Veterinarian: **Dr. Marcus Webb, DVM, DACVECC**

SOAP NOTE — EMERGENCY PRESENTATION

Note Type:	SOAP	Date of Visit:	2024-06-20
Patient:	Nori	Owner:	Aiko Tanaka
Species:	Feline	Breed:	Domestic Shorthair
DOB:	2021-04-03	Weight:	4.1 kg

S — SUBJECTIVE

Owner Aiko Tanaka presents Nori, a 3-year-old female spayed Domestic Shorthair, via emergency walk-in. Owner reports the patient vomited 7 times over the past 8 hours. Vomitus described as bilious, then unproductive retching. Patient has not eaten since yesterday morning. Owner states Nori was playing with a linear piece of string approximately 30 cm in length yesterday afternoon and it is no longer visible. Patient is lethargic and hunched. No diarrhoea reported. Last defecation 36 hours ago. No outdoor access. No known toxin exposure.

O — OBJECTIVE

General: QAR (Quiet, Alert, Responsive). Posture hunched, guarded abdomen. BCS 4/9. Weight: 4.1 kg (4.4 kg at last visit 3 months ago — 6.8% weight loss).

Vital Signs:

Parameter	Value	Reference Range	Status
Temperature	38.9 C	38.0–39.5 C	Normal
Heart rate	196 bpm	120–200 bpm	Elevated
Respiratory rate	28 bpm	16–40 bpm	Normal
Mucous membranes	Pale pink, tacky	Pink, moist	Mildly dehydrated
CRT	2.5 sec	< 2 sec	Prolonged

CV: Tachycardia. Resp: Clear. Abdomen: Tense, painful on cranial palpation; plication of small intestinal loops palpated. Oral exam: linear foreign material visible anchored under the tongue. Skin turgor: mildly reduced.

A — ASSESSMENT

1. Linear foreign body ingestion — string anchored at tongue base with suspected intestinal plication. High suspicion for gastrointestinal obstruction. 2. Acute vomiting — secondary to GI obstruction. 3. Mild dehydration (estimated 6–7%) — secondary to vomiting and reduced intake. 4. Tachycardia — likely pain- and dehydration-related; monitor. Differential: intestinal perforation cannot be excluded pending imaging.

P — PLAN

- Abdominal radiographs (3 views): confirm plication pattern consistent with linear FB. Imaging findings support surgical intervention. - IV access established (22G cephalic catheter). IV fluid resuscitation: LRS at 10 mL/kg/hr. - Anti-emetic: Maropitant (Cerenia) 1 mg/kg IV. - Analgesia: Buprenorphine 0.02 mg/kg IV. - Pre-surgical bloodwork: CBC + full chemistry panel STAT. - Surgical consent obtained from owner. Exploratory laparotomy and foreign body removal scheduled immediately (on-call surgery team). Gastrotomy and/or enterotomy anticipated. - Owner informed of guarded prognosis pending surgical findings. Risk of intestinal perforation and septic peritonitis discussed.

EXPECTED CODING OUTPUT (test validation reference)

Expected Code	System	Category	Confidence Tier
T19.8XXA	ICD-10-CM	DIAGNOSIS	HIGH
K56.49	ICD-10-CM	DIAGNOSIS	HIGH
R11.10	ICD-10-CM	SYMPTOM	HIGH
E86.0	ICD-10-CM	DIAGNOSIS	HIGH
R00.0	ICD-10-CM	SYMPTOM	MEDIUM
0D9E0ZZ	ICD-10-PCS	PROCEDURE	HIGH

Test assertion: requires_review = false | intestinal perforation differential NOT coded as confirmed diagnosis (negation rule CR-08) | primary_diagnosis = T19.8XXA (linear foreign body)